INFORMATION TO KEEP WITH YOU

My complete treatment handbook

CAP: advanced salivary gland cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00615
Version 2

Day 1 every 21 days; up to 6 cycles

Treatment stages

01 **Chemotherapy course**
Day 1 every 21 days; up to 6 cycles

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

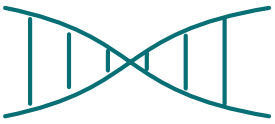
How the medicines work

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

CYTOTOXIC CHEMOTHERAPY

Cyclophosphamide

Intravenous chemotherapy



Alkylates and damages deoxyribonucleic acid (the cell's genetic material) (DNA)

Cyclophosphamide is activated in the body into compounds that damage DNA. Bladder irritation is one reason for a clear hydration plan.

Activated in the body → DNA damaged → Division reduced

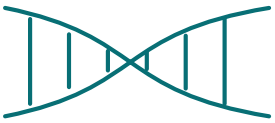
What to watch for

Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.

CYTOTOXIC CHEMOTHERAPY

Doxorubicin

Intravenous chemotherapy



Damages DNA and interferes with repair

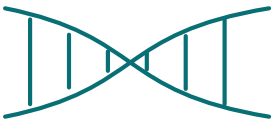
Doxorubicin interacts with DNA and topoisomerase II, disrupting copying and repair. Its heart and tissue effects require particular care.

DNA interaction → Repair disrupted → Division reduced

CYTOTOXIC CHEMOTHERAPY

Cisplatin

Intravenous chemotherapy



Cross-links cancer-cell DNA

Cisplatin forms links within DNA that impede copying. The kidneys, hearing and nerves can also be affected.

DNA cross-links → Copying blocked → Division reduced

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Research supports a treatment discussion; it cannot predict one person’s outcome.

The NCCP CAP regimen draws on small salivary gland cancer studies. Salivary gland cancers include different histologies, and this evidence does not provide a reliable personalised survival gain. Discuss the aim of symptom control, chance of tumour response, burdens of treatment and histology-specific alternatives.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

CAP: advanced salivary gland cancer

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY 1 EVERY 21 DAYS; UP TO 6 CYCLES

Chemotherapy course

Up to 6 cycles

Doxorubicin 50 mg/m², cisplatin 50 mg/m² and cyclophosphamide 500 mg/m² on Day 1, repeated every 21 days for up to 6 cycles, stopping earlier for progression or unacceptable side effects.

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00615 v2: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests and ECG. Heart-pumping assessment with an echo or MUGA scan if over 65 or clinically indicated; doxorubicin requires LVEF above 50%. Hearing assessment and measured kidney clearance when indicated.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

CAP: advanced salivary gland cancer: treatment and checks

Advanced salivary gland cancer, after specialist review of histology, treatment choices and goals.

Doxorubicin 50 mg/m², cisplatin 50 mg/m² and cyclophosphamide 500 mg/m² on Day 1, repeated every 21 days for up to 6 cycles, stopping earlier for progression or unacceptable side effects.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests and ECG. Heart-pumping assessment with an echo or MUGA scan if over 65 or clinically indicated; doxorubicin requires LVEF above 50%. Hearing assessment and measured kidney clearance when indicated.	Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.

Care / check	Before starting	Treatment visits	Between visits	Next cycle	After treatment
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.	Keep a current dose and symptom record.	Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1



Treatment and review

Day 1 every 21 days; up to 6 cycles

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / **act:** Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before starting

Full blood count, kidney and liver tests and ECG. Heart-pumping assessment with an echo or MUGA scan if over 65 or clinically indicated; doxorubicin requires LVEF above 50%. Hearing assessment and measured kidney clearance when indicated.

STEP 2

Taking or receiving treatment

Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.

STEP 3

Tests and reviews

Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Your exact treatment schedule

Tests and preparation

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your 21-day cycle.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
CAP infusion and fluids	 **Day 1** Doxorubicin 50 mg/m², cisplatin 50 mg/m² and cyclophosphamide 500 mg/m² on Day 1, repeated every 21 days for up to 6 cycles, stopping earlier for progression or unacceptable side effects.
Recovery and safety checks	 **Days 2–21** Report symptoms throughout the cycle; take prescribed support medicines.

Treatment dates depend on safety checks and the individual prescription. Side effects can occur on any day.

The next cycle starts 21 days after this one, if your team confirms it.

> Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1: Day 1: three medicines and fluids

Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line. Tell the nurse immediately about infusion-site pain.

Day 2–21: Between visits

Follow the written anti-sickness and fluid plan. Blood counts can fall after treatment; no day is guaranteed safe. Keep the next blood-test and clinic dates; call urgently if unwell.

Support around the medicines

1 Preventing and treating sickness

CAP is high sickness risk: take the full prescribed prevention and rescue plan, including any locally prescribed stomach protection.

2 Medicine-specific support

3 Mouth care, nutrition and daily life

Follow the unit's mouth-care plan. Report pain, poor intake or weight loss early; ask for dietetic support. Pace activity, rest and work around symptoms.

4

Medicine and interaction review

Bring all prescription, non-prescription and herbal medicines. Check with the oncology pharmacist before starting or stopping anything.

Your diagnosis and treatment setting

Specialist tests

There is no universal serum tumour-marker panel for this regimen. Histology, molecular testing and scans are chosen by the salivary gland multidisciplinary team.

Treatment aims and choices

Review and alternatives

The team reviews the cancer, side effects and your preferences. Scans follow your individual treatment plan; this regimen does not set one universal scan interval. Discuss suitable alternatives, supportive care and the option of declining treatment.

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

Try explaining the plan back: Explain when you take or receive treatment.,Name the tests and next review date.,Show how to contact the team urgently.,Explain what to do if a dose is missed or treatment is held.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1: CAP

Active medicines: Cyclophosphamide + Doxorubicin + Cisplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.

Medicine	Dose / route	When
Ondansetron	16 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	12 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Aprepitant	80 mg PO OD	Days 2-3 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	8 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Doxorubicin is given slowly into a vein over about 15 minutes, cisplatin over 2 hours, then cyclophosphamide over 30 minutes. Cisplatin requires prescribed intravenous fluids before and after treatment, so the visit takes longer than the drug infusion times. Tell the nurse immediately about burning, pain, swelling or leakage around the line.
- Full blood count, kidney and liver tests during treatment. Review hydration, blood salts, hearing, tingling or numbness, heart symptoms, mouth care and prior toxicities before further doses. Record lifetime anthracycline exposure; the source doxorubicin limit is 450 mg/m².
- Follow the unit's cisplatin hydration plan and individual fluid target. The source advises 2–3 litres on Day 1 to reduce bladder irritation from cyclophosphamide; the team must adapt this if heart or kidney problems restrict fluids. Report blood in urine or painful urination promptly.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Fatigue and neurological symptoms

CAP

Balance activity and rest. Sudden confusion, seizure, severe headache or a change in vision needs urgent help.

2 Mouth, eating and swallowing

CAP

Report ulcers, pain, dry mouth, difficulty swallowing or weight loss early. Ask for oral care, dietetic and swallowing support.

3 Blood counts and infection

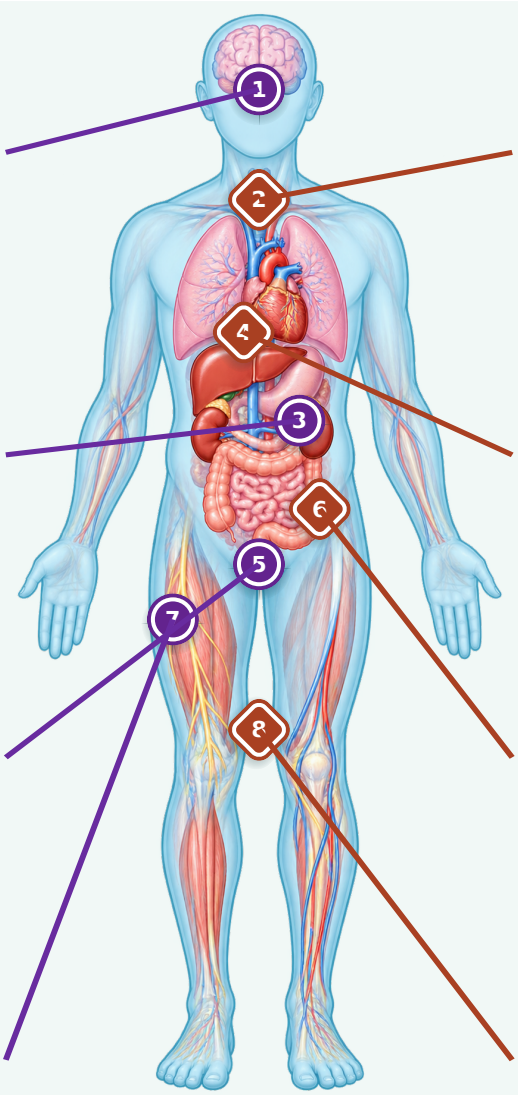
CAP

Fever, shivering, sudden illness, unusual bruising or bleeding needs urgent oncology advice.

4 Heart, hearing and nerves

Doxorubicin / cisplatin

Report breathlessness, chest symptoms, hearing changes or tingling promptly. Heart, hearing and nerve injury can persist.



5 Sickness and bowels

CAP

Use prescribed support. Report persistent diarrhoea or vomiting, inability to drink, blood or severe abdominal pain.

6 Kidneys and fluid balance

CAP

Report reduced urine, swelling or poor fluid intake; keep blood and urine tests.

7 Hair and infusion site

Doxorubicin / cyclophosphamide

Hair loss may occur. Tell the nurse immediately about burning, pain, swelling or leakage at the infusion site.

8 Fertility and pregnancy

CAP

Discuss fertility preservation, contraception, pregnancy and breastfeeding before starting.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for fever, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink. Severe chest pain, severe breathing difficulty, collapse or a seizure: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00615 v2: official regimen**
   https://healthservice.hse.ie/documents/6994/615_V2_Cyclodoxcis.pdf
2. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
3. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
4. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
5. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf